

MEDICAL REPORT

Patient Name: Maria Thompson
Date of Birth: 09/18/1956
Medical Record Number (MRN): 778899001
Gender: Female
Age: 69
Date of Service: September 18, 2025
Provider: Helen Carter, MD
Visit Type: Established Patient Visit

CHIEF COMPLAINT

Follow-up for diabetes, shortness of breath with activity, and kidney function monitoring.

HISTORY OF PRESENT ILLNESS

Maria Thompson is a 69-year-old female presenting today for a scheduled chronic care follow-up. She reports that her home glucose readings have been running high over the past several months, usually between 170 and 220 mg/dL before breakfast. She admits that she has missed some evening doses of her medication and has had difficulty following a low-carbohydrate diet since her husband became ill.

She reports increased fatigue and waking at night to urinate two to three times. She denies nausea, vomiting, abdominal pain, or symptoms of hypoglycemia. She reports mild burning and tingling in both feet, worse at night, but denies open wounds or recent infections.

The patient also reports shortness of breath when walking more than one block and occasional swelling in both ankles by the evening. She denies chest pain, syncope, or palpitations. She sleeps with two pillows but denies waking up gasping for air. She states she has been taking furosemide as prescribed but sometimes skips doses when leaving the house.

She has a known diagnosis of chronic kidney disease related to diabetes and hypertension. She has not seen nephrology in the past year. She denies flank pain, hematuria, dysuria, or decreased urine output.

PAST MEDICAL HISTORY

- Type 2 diabetes mellitus diagnosed in 2011.
- Chronic kidney disease, previously stage 3b.
- Congestive heart failure diagnosed in 2022.
- Essential hypertension.
- Hyperlipidemia.
- History of chronic obstructive pulmonary disease, ICD-10 J44.9, listed in old records only. Patient denies chronic cough, wheezing, inhaler use, oxygen use, or COPD exacerbation. No COPD assessment or treatment was performed today.
- History of major depressive disorder, recurrent, ICD-10 F33.1, listed in prior chart problem list. Patient denies depressed mood, anhedonia, suicidal ideation, sleep disturbance related to mood, or current behavioral health treatment. No depression medication adjustment or mental health plan was addressed today.

MEDICATIONS

- Metformin ER 500 mg by mouth twice daily.
- Empagliflozin 10 mg by mouth daily.
- Lisinopril 20 mg by mouth daily.
- Furosemide 40 mg by mouth daily.
- Metoprolol succinate 50 mg by mouth daily.
- Atorvastatin 40 mg by mouth nightly.
- Aspirin 81 mg by mouth daily.

ALLERGIES

No known drug allergies.

SOCIAL HISTORY

Former smoker, quit in 2010 after approximately 20 pack-years. Denies current tobacco use. Drinks wine socially, about one to two glasses weekly. Lives with spouse. Independent with activities of daily living.

REVIEW OF SYSTEMS

Constitutional: Reports fatigue. Denies fever, chills, or unintentional weight loss.

Cardiovascular: Reports exertional dyspnea and ankle swelling. Denies chest pain or palpitations.

Respiratory: Denies cough, wheezing, sputum production, or hemoptysis.

Endocrine: Reports high home glucose readings and increased thirst.

Genitourinary: Reports nocturia. Denies dysuria or hematuria.

Neurologic: Reports burning and tingling in both feet. Denies weakness, falls, or new focal deficits.

Psychiatric: Denies depressed mood, anxiety, suicidal ideation, or hallucinations.

VITAL SIGNS

Blood Pressure: 146/84 mmHg

Heart Rate: 78 bpm, regular

Respiratory Rate: 16 breaths per minute

Temperature: 98.4°F

Oxygen Saturation: 96% on room air

Height: 5 ft 4 in

Weight: 188 lb

BMI: 32.3 kg/m²

PHYSICAL EXAMINATION

General: Alert, pleasant female in no acute distress.

Cardiovascular: Regular rate and rhythm. S1 and S2 present. No murmur. Trace bilateral ankle edema.

Respiratory: Lungs clear to auscultation bilaterally. No wheezing, rhonchi, or crackles noted today.

Abdomen: Soft, non-tender, non-distended. Normal bowel sounds.

Extremities: Trace edema at both ankles. No cyanosis. Peripheral pulses palpable.

Feet: Skin intact bilaterally. No ulcers, erythema, drainage, or calluses. Decreased monofilament sensation at bilateral great toes.

Neurologic: Alert and oriented. Strength 5/5 in upper and lower extremities. Gait steady.

Psychiatric: Normal mood and affect. Cooperative. Thought process linear.

LABORATORY AND DIAGNOSTIC RESULTS

Labs reviewed from September 16, 2025:

- Hemoglobin A1c: 8.9%.
- Fasting glucose: 204 mg/dL.
- Creatinine: 1.8 mg/dL.
- eGFR: 31 mL/min/1.73m².
- Urine albumin-to-creatinine ratio: 122 mg/g.
- Potassium: 4.8 mEq/L.
- Sodium: 139 mEq/L.
- LDL cholesterol: 94 mg/dL.
- BNP: 615 pg/mL.

Chest X-ray from September 16, 2025:

Mild cardiomegaly without focal infiltrate. No pleural effusion.

ASSESSMENT

1. Type 2 diabetes mellitus with diabetic chronic kidney disease.

The patient's diabetes is uncontrolled with A1c 8.9% and fasting glucose 204 mg/dL. She also has diabetic kidney disease with elevated urine albumin-to-creatinine ratio and reduced eGFR.

Medication adherence and diet were reviewed in detail today. Empagliflozin will be continued due to renal and cardiovascular benefit. Metformin will be reduced due to kidney function and risks were discussed.

2. Chronic kidney disease, stage 3b.

Current eGFR is 31 mL/min/1.73m² with creatinine 1.8 mg/dL. Kidney function has declined compared with prior eGFR of 39 six months ago. Patient counseled to avoid NSAIDs, maintain hydration, and monitor blood pressure. Referral to nephrology placed today.

3. Chronic systolic congestive heart failure.

Patient reports exertional dyspnea and evening ankle swelling. BNP is elevated at 615 pg/mL and chest X-ray shows mild cardiomegaly. She is currently taking furosemide and metoprolol. She admits occasionally skipping diuretic doses, so adherence was reinforced. No acute respiratory distress today.

4. Severe obesity due to excess calories with BMI 32.3 and serious comorbidity.

BMI today is 32.3 kg/m². Obesity is contributing to diabetes, hypertension, and heart failure risk. Nutrition counseling was provided, and a referral to a dietitian was placed for structured weight management.

PLAN

Diabetes with CKD:

- Continue empagliflozin 10 mg daily.
- Reduce metformin ER to 500 mg once daily due to eGFR 31.
- Start semaglutide 0.25 mg subcutaneous weekly after discussion of benefits, risks, and side effects.
- Patient instructed to check fasting glucose daily and bring log to next visit.
- Counseled on reducing sweetened beverages and refined carbohydrates.

Chronic kidney disease stage 3b:

- Referral placed to nephrology.
- Repeat CMP and urine albumin-to-creatinine ratio in 3 months.
- Avoid NSAIDs and contrast exposure when possible.
- Continue lisinopril for renal protection and blood pressure control.

Chronic systolic heart failure:

- Continue furosemide 40 mg daily and metoprolol succinate 50 mg daily.
- Reinforced daily weights and low-sodium diet.
- Patient advised to call for weight gain greater than 3 lb in one day or 5 lb in one week.
- Follow-up with cardiology recommended within 4 weeks.

Obesity:

- Dietitian referral placed.
- Encouraged 150 minutes per week of low-impact activity as tolerated.
- Weight loss goal of 5% body weight over 6 months discussed.

Past history items not addressed today:

- COPD appears in historical problem list only. No symptoms, exam findings, medication management, testing, or treatment plan for COPD today.
- Major depressive disorder appears in historical problem list only. Patient denies active symptoms, and no evaluation or treatment plan was performed today.

Follow-up:

Return to clinic in 3 months or sooner for worsening shortness of breath, swelling, hypoglycemia, chest pain, or reduced urine output.

Provider Signature: Helen Carter, MD

Signed: September 18, 2025